SOP Number & Full Title:	SOP ID: 3872 Anaesthesia SOP 14: The use of TEG in Major Obstetric Haemorrhage
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Audience	Anaesthetists and ODPs / Anaesthetic practitioners working in Obstetric Haemorrhage. Obstetric doctors and midwives.

Introduction

This SOP outlines the management algorithm that should be followed when using the point of care TEG6s® Haemonetics® Analyser in the obstetric population (over 20 week's gestation) at NUH.

Details of how to correctly take the blood sample and run the test are outlined elsewhere, and are unchanged.

TEG6s is a point of care testing looking at the ability of the blood to clot – it gives faster results than a laboratory sample, allowing clinicians to more accurately guide the administration of blood and blood products. This will reduce the risk to women from unnecessary transfusions, and optimise their clotting profile. The algorithm has been created using blood samples from obstetric patients at NUH and produced in liaison with Haematology.

Procedure

This TEG algorithm is to be used only in **major obstetric haemorrhage**.

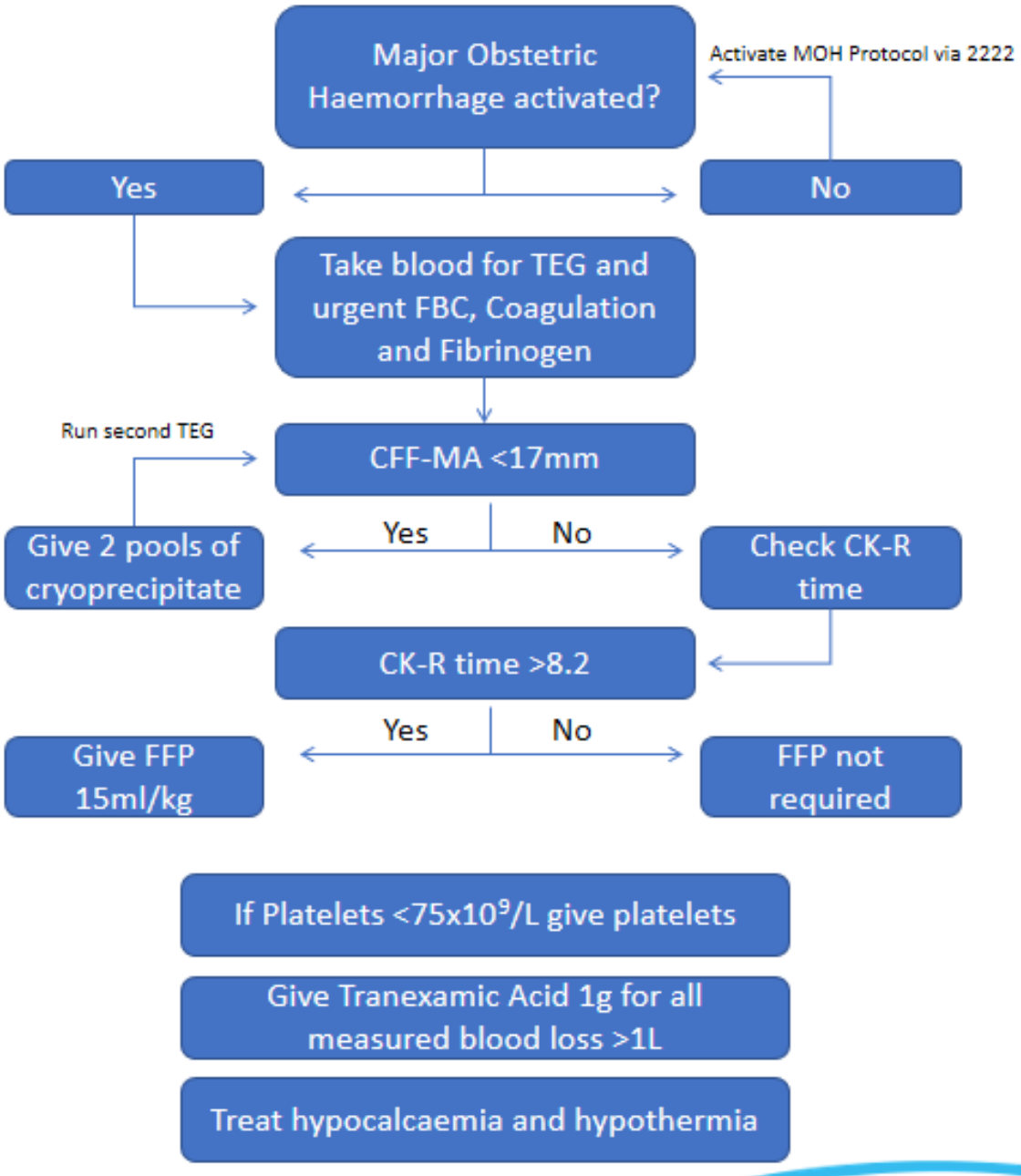
After the sample has been analysed, the results are displayed on the TEG6s machine and on the TEG manager on desktop computers.

The following flowchart should be followed to guide blood product administration in obstetric major haemorrhage. This flow chart is available in obstetric theatres.

Obstetric TEG Treatment Algorithm

NHS
Nottingham University Hospitals
NHS Trust

For Major Obstetric Haemorrhage Only



We Listen We Care

References:

1. T.C.D. Roberts, L. De Lloyd, S.F. Bell et al. Utility of viscoelastography with TEG 6s to direct management of haemostasis during obstetric haemorrhage: a prospective observational study. International Journal of Obstetric Anaesthesia. 2021; 47: 103192

2. 2. P. A. Gurbel, K. P. Bliden, U. S. Tantry et al. First report of the point-of-care TRG: A technical validation study of the TEG-6S system. Platelets, DOI 10.3109/09537104.2016.1153617

3. 3. B. Macafee, J. P. Campbell, K. Ashpole et al. Reference ranges for thromboelastography (TEG) and traditional coagulation tests in term parturients undergoing caesarean section under spinal anaesthesia. Anaesthesia. 2012; 67: 741-747